

Missed first post-discharge follow-up appointment due to lack of automated tracking and unclear ownership

People

Case Manager cognitive overload (40-80 patients)

Reliance on memory for follow-up tasks

Unclear ownership between inpatient and outpatient teams

Process

Lack of required confirmation before discharge

No standardized discharge-to-appointment workflow

Follow-up depends on manual outreach

Technology

No automated alert for missing appointment

EHR (like Epic Systems) not configured to enforce scheduling

No real-time integration between inpatient and outpatient systems

Discharge info not clearly communicated to outpatient team

Patient is the "messenger" between providers

Appointment details unclear or delayed

Communication

High patient volume and time pressure

Insurance constraints delaying scheduling

Fragmented healthcare ecosystem

Environment

Case manager responsible without authority

No enforced accountability for follow-up completion

Incentives focused on discharge speed, not continuity

Policy / Management

Primary Root Cause: Over-reliance on manual, human-dependent coordination instead of automated, system-triggered workflows.